

LABORATORY MEDICINE PROGRAM

UUID: CE03F0A7-D71D-4896-9401-2157AC622BFE
TCGA-EM-A308-01A-PR

Redacted



Surgical Pathology Consultation Report

Patient Name:

MRN:

DOB:

Gender:

HCN:

Ordering MD:

Copy To:

Service:

Visit #:

Location:

Facility:

Accession #:

Collected:

Received:

Reported:

Specimen(s) Received

1. Parathyroid: Left Biopsy, Lower
2. Thyroid: Total thyroid tie upper pole left lobe
3. Parathyroid: Left lower parathyroid specimen #2

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
IPAA Discrepancy		☒
Prior Malignancy History		☒
Qual/Synchronous Primary Noted		☒
see is (circle):	QUALIFIED	DISQUALIFIED
review of info:	2/5/12	12/1/12

Diagnosis

1. No pathological diagnosis: Soft tissue, site listed as "left lower parathyroid", biopsy.
2. Multifocal papillary carcinoma, dominant follicular variant 1.3 cm, right; follicular nodular disease with extensive degeneration and reactive atypia; focal mild reactive C-cell hyperplasia: Thyroid
No pathological diagnosis: Lymph nodes, 3, left perithyroidal
No pathological diagnosis: Parathyroid, left superior
No pathological diagnosis: Thymus
-Total thyroidectomy specimen. See Microscopic Description.
3. No pathological diagnosis: Parathyroid, site listed as "left lower parathyroid specimen #2", biopsy. See Microscopic Description.

Synoptic Data

Procedure: Total thyroidectomy
Received: Fresh
Specimen Integrity: Intact
Specimen Size: Right lobe:
6.0 cm
3.1 cm
1.8 cm
Left lobe:
6.3 cm
2.9 cm
2.0 cm
Isthmus +/- pyramidal lobe:
2.2 cm
1.1 cm
0.6 cm
Specimen Weight: 38.7 g

1CD-0-3
carcinoma, papillary, follicular variant
8340/3
Site: thyroid, NOS C73.9
w
2/5/12

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Tumor Focality: Multifocal, bilateral
----- DOMINANT TUMOR -----
Tumor Laterality: Right lobe
Tumor Size: Greatest dimension: 1.3cm
Additional dimension: 1.1 cm
Additional dimension: 1.1 cm
Histologic Type: Papillary carcinoma, follicular variant
Follicular architecture
Other architecture: focal trabecular architecture.
Classical cytomorphology
Histologic Grade: Not applicable
Margins: Margins uninvolved by carcinoma
Tumor Capsule: Totally encapsulated
Tumor Capsular Invasion: Present, extent minimal
Lymph-Vascular Invasion: Not identified
Perineural Invasion: Not identified
Extrathyroidal Extension: Not identified
----- SECOND TUMOR -----
Tumor Laterality: Left lobe
Tumor Size: Greatest dimension: 0.8cm
Histologic Type: Papillary carcinoma, follicular variant
Follicular architecture
Classical cytomorphology
Histologic Grade: Not applicable
Margins: Margins uninvolved by carcinoma
Tumor Capsule: Totally encapsulated
Tumor Capsular Invasion: Not identified
Lymph-Vascular Invasion: Not identified
Perineural Invasion: Not identified
Extrathyroidal Extension: Not identified
TNM Descriptors: m (multiple primary tumors)
Primary Tumor (pT): pT1b: Tumor more than 1 cm but not more than 2 cm in greatest dimension, limited to the thyroid
Regional Lymph Nodes (pN): pN0: No regional lymph node metastasis
Number of regional lymph nodes examined: 3
Number of regional lymph nodes involved: 0
Distant Metastasis (pM): Not applicable
Additional Pathologic Findings: Adenomatoid nodule(s) or Nodular follicular disease
Parathyroid gland(s), within normal limits
C-cell hyperplasia
Other: Multiple additional papillary microcarcinomas identified, measuring up to 0.3 cm, in right and left lobes. Thymus with no pathological diagnosis.
Ancillary Studies: Type: IHC
Results: Calcitonin stains foci of mild reactive C-cell hyperplasia. The dominant nodule is positive for thyroglobulin and negative for calcitonin, monoclonal CEA and chromogranin.

*Pathologic Staging is based on AJCC/UICC TNM, 7th Edition

Electronically verified by:

Gross Description

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1. The specimen labelled with the patient's name and as "left biopsy lower" consists of a piece of tissue that measures 0.3 x 0.3 x 0.2 cm. It is frozen for intraoperative consultation.

1A frozen tissue resubmitted

2. The specimen labeled with the patient's name and as "total thyroid tie upper pole left lobe" consists of a thyroid gland that is oriented with suture on upper left lobe and weighs 38.7 g. The right lobe measures 6 cm SI x 3.1 cm ML x 1.8 cm AP, the left lobe measures 6.3 cm SI x 2.9 cm ML x 2 cm AP and the isthmus measures 2.2 cm SI x 1.1 cm ML x 0.6 cm AP. The external surfaces have fibrous adhesions and are painted with blue dye. No parathyroid glands or lymph nodes are identified grossly. The mid to lower pole of the right lobe contains 3 delineated nodule, the largest homogeneous and solid, T1, measures 1.3 cm SI x 1.1 cm ML x 1.1 cm AP. The upper mid to lower pole of the left lobe contains multiple delineated nodules, the largest partly solid and partly colloidal, T2, measures 1.0 cm SI x 1.2 cm ML x 1.1 cm AP. The remainder of the thyroid tissue is grossly unremarkable. Sections of the 2 largest nodules and normal tissue are stored frozen. The remainder of the specimen is submitted as follows:

2A-J right lobe, superior to inferior

2K isthmus

2L-V left lobe, superior to inferior

3. The specimen labelled with the patient's name and as "left lower parathyroid specimen #2" consists of a piece of tissue that measures 0.2 x 0.2 x 0.2 cm. It is frozen for intraoperative consultation.

3A frozen tissue resubmitted

Quick Section Diagnosis

1. Parathyroid (Left, lower): Dx QS1A, 1A deeper: Fibroadipose tissue, no definite parathyroid tissue identified (deeper levels done).

3. Parathyroid (Left, lower): Dx QS3A: Parathyroid tissue present.

Microscopic Description

2. There is focal very mild C-cell hyperplasia which represents a reactive phenomenon due its exclusive association with underlying prominent follicular epithelial cell proliferations.

3. Parathyroid tissue is confirmed to be present in the frozen section slides; however upon sectioning from the paraffin block, the tissue is exhausted and only soft tissue remains.